GAMSA · Gender-Affirming Medicines South Australia

Competency assessment checklist

Ventrogluteal intramuscular administration of testosterone undecanoate 1000 mg/4 mL

DOCUMENT ID	VERSION	EFFECTIVE	REVISED	NEXT REVIEW
GAMSA-TU-COMP	2.5	5 October 2025	6 November 2026	24 October 2027, or on any change to the Product Information

Current version

Prepared by Theodore South, BPharm (Hons), registered pharmacist, Adelaide. Questions or corrections: gamsaenquiries@gmail.com. A template for adaptation to local policy, offered freely; not endorsed by any health service. Clinics remain responsible for their own procedures, credentialling and emergency arrangements. Where this document and the Product Information differ, the Product Information prevails [1].

Assesses competency in the safe preparation and administration of testosterone undecanoate 1000 mg/4 mL by deep intramuscular injection at the ventrogluteal site. Every criterion must be demonstrated under direct observation (2a is a recommended step: tick N/A if pregnancy is not possible); the SOP and training guide sets the standard expected. Complete after at least three supervised injections.

Clinician	Position	Assessor	Date

Competency criteria

#	Criterion	Source	Competent	Comments
1	Confirms patient identity with at least three approved identifiers (not sex or gender) and matches them to the prescription, using the name the person uses; confirms allergies, contraindications and informed consent, including any prior reaction to an oil-based injection	[1][12][13]	☐ Yes ☐ No	
2	Screens for current chest infection or breathlessness, previous POME, and bleeding or coagulation disorder or anticoagulant use; knows to defer or refer	[1][11]	☐ Yes ☐ No	
2a	Recommended: if the patient could become pregnant, checks that a pregnancy test has been done before the injection; knows testosterone is contraindicated in pregnancy	[1][12]	☐ Yes ☐ No ☐ N/A	
3	Confirms product, dose (1000 mg in 4 mL), expiry and batch number	[1][2]	☐ Yes ☐ No	
4	Prepares equipment: 18G drawing-up needle, 20–22G injection needle of suitable length (38 mm; 50 mm if BMI over 30), 5 mL syringe, antiseptic, dressing, sharps container	[5][10]	☐ Yes ☐ No	
4a	Cleans hands at the 5 Moments (before touching the patient, before the injection, after the injection or any blood exposure, after touching the patient and their surroundings) and puts on gloves before the injection	[5][14]	☐ Yes ☐ No	
5	Confirms emergency equipment is available and in date: oxygen, adrenaline, monitoring, means of calling 000	[8]	☐ Yes ☐ No	
6	Positions the patient and landmarks the ventrogluteal site correctly (greater trochanter, ASIS, iliac crest; centre of the V)	[4][5]	☐ Yes ☐ No	
7	Inserts the needle at 90° to a depth that reaches muscle; aspirates, and knows to withdraw and re-site if blood returns	[5][10]	☐ Yes ☐ No	

#	Criterion	Source	Competent	Comments
8	Injects the full 4 mL very slowly, over approximately 2 minutes	[1][3][10]	☐ Yes ☐ No	
9	Withdraws the needle, applies gentle pressure without massage, applies dressing, disposes of sharps immediately	[5]	☐ Yes ☐ No	
10	Observes the patient during and immediately after the injection, and for at least 30 minutes under this procedure (POME typically presents within 30 minutes; the PI gives no fixed time); books the next appointment	[1][3][12]	☐ Yes ☐ No	
11	Describes the signs of POME and of anaphylaxis, and the immediate management of each, including adult adrenaline dose, route and repeat	[6][7][8]	☐ Yes ☐ No	
12	Documents product, batch, expiry, dose, side, needle, clinician, observation outcome and next due date; knows how to report to the TGA (AEMS)	[1][9]	☐ Yes ☐ No	
12a	Describes what to do after a needlestick or blood splash: wash or rinse straight away, tell the manager and record it, get medical advice straight away	[15][16]	☐ Yes ☐ No	

Assessment outcome

☐ **Competent**	☐ **Requires further supervised practice**	☐ **Not yet competent**

Assessor comments and plan:

Assessor signature	Name (print)	Date

Clinician signature	Name (print)	Date

Sources Numbered as in the SOP and training guide, where full references are given: [1] Reandron 1000 Australian PI, Aspen, rev. Dec 2025 · [2] Reandron 1000 ARTG 213942 · [3] Reandron 1000 NZ data sheet, Dec 2025 · [4] SA Health ventrogluteal brochure · [5] SESLHD GL/076 v2.6, 2025 · [6] Pastuszak 2020 (POME safety analysis) · [7] Adam 2023 (POME review) · [8] ASCIA anaphylaxis guideline, October 2026 · [9] TGA AEMS adverse event reporting · [10] UK administration guidance (Liconsa), 2024 · [11] Testosterone ADVZ 1000 Australian PI · [12] AusPATH Australian Standards of Care, 2026 · [13] Canberra Health Services CHS25/344 · [14] ACSQHC hand hygiene guide, 2023 · [15] NSW Health GL2024_002, 2024 · [16] NIOSH 2012-123. Current package: https://gamsa.au/medicines/testosterone-undecanoate

Amendment history (GAMSA-TU-COMP)

Version	Date	What changed
2.0–2.3	2–24 October 2026	GAMSA edition (first prepared 5 October 2025); criterion 2a; observation at least 30 minutes; new layout; fillable form fields; document control lines.
2.4	31 October 2026	Contact email for questions or corrections added; nothing else changed.
2.5	6 November 2026	Criterion 1: three identifiers; new criteria 4a (hand hygiene) and 12a (needlestick and blood splash); sources [13] to [16].